NCCP 00588 · clinician draft

Olaparib · adjuvant breast treatment

Breast adjuvant treatment only · NCCP00588h. These materials cover olaparib after treatment for high-risk early breast cancer. Other cancers use different eligibility, duration and follow-up; use the matching NCCP indication and your treating team's plan.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Olaparib · adjuvant breast treatment · supportive-care review

Active medicines: Olaparib (Lynparza)

Emetogenic risk: Oral moderate–high

No routine prophylactic antiemetic order is populated for this phase. Agree an individual symptom-treatment plan where needed.

Additional phase-specific support

- NCCP classifies oral olaparib as moderate-to-high emetic risk. Agree an anti-sickness plan under local policy, and seek review if oral intake falls. The SmPC describes early/intermittent symptoms; do not automatically copy an intravenous chemotherapy antiemetic cycle.
- Aprepitant is a moderate CYP3A inhibitor in NCCP 00588 and can increase olaparib exposure. The prescriber/ pharmacist should choose and reconcile supportive medicines; never add an antiemetic from another regimen without checking.
- Anaemia or other low counts can lead to interruption, dose adjustment, transfusion or additional investigation. There is no universal routine G-CSF course for every person on this regimen.
- Review all prescriptions, over-the-counter drugs and herbal products. Strong/moderate CYP3A inhibitors or inducers can change exposure. Avoid grapefruit products and seek advice before starting anything new.
- NCCP allows consideration of a proton pump inhibitor under local policy. This is symptom- and patient-dependent, rather than a compulsory medication for everyone.
- Agree an oral antiemetic prevention / rescue plan before treatment. Continuous oral treatment does not use the Day1–4 IV chemotherapy template. Confirm agent, dose, frequency, duration and interactions; no default daily steroid order.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00588 v5b · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-20 · SACTCheck v0.84.9 · Regimen-level drafts; prescriber and oncology-pharmacy verification required